

SDAI · HIMSHIKHAR 2026 · TEAM-GPT

ClinicFlow

Clinic Appointment and Intake Summary System

Healthcare administration system

Team

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clinicappointment23.netlify.app

THE PROBLEM

Clinic intake is manual, slow, and easy to lose track of



Missing information is caught late

Often not noticed until the patient is already in the room.



Staff re-read full forms every visit

No summary exists — every intake form is read cold.

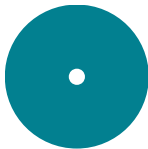


Follow-ups fall through the cracks

Missing documents or unclear answers aren't tracked centrally.

ClinicFlow gives staff one dashboard with AI-drafted summaries, automatic missing-field flags, and a live follow-up tracker — removing the manual overhead without touching any clinical decision.

Starter data — synthetic, schema-driven



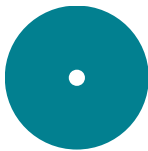
`patients.csv`

Patient identity & contact records



`appointments.csv`

Appointment slots, date/time, status



`intake_forms.csv`

Submitted intake form responses

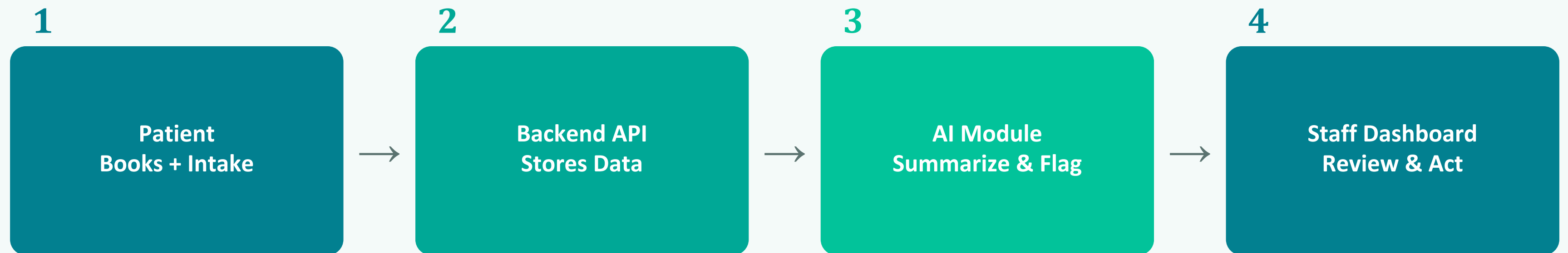


`followups.csv`

Follow-up items flagged by staff / AI

[Add: number of records created per file — spec recommends 30–100 realistic records. All data is synthetic / fictional, not real patient information.]

System workflow: from booking to staff action



Every AI output is labeled as an administrative draft only — not medical advice. Staff make every clinical decision.

Three bounded, administrative AI tasks



Intake Summarization

Condenses patient intake text into a short staff-facing draft.



Missing-Field Detection

Checks submitted forms against required fields and flags gaps.



Follow-Up Drafting

Generates a short, editable suggested follow-up reminder.

Why AI here: summarizing, gap-checking, and drafting reminders are judgment-light, repetitive tasks — well suited to an LLM. Diagnosis and treatment stay entirely with clinical staff.

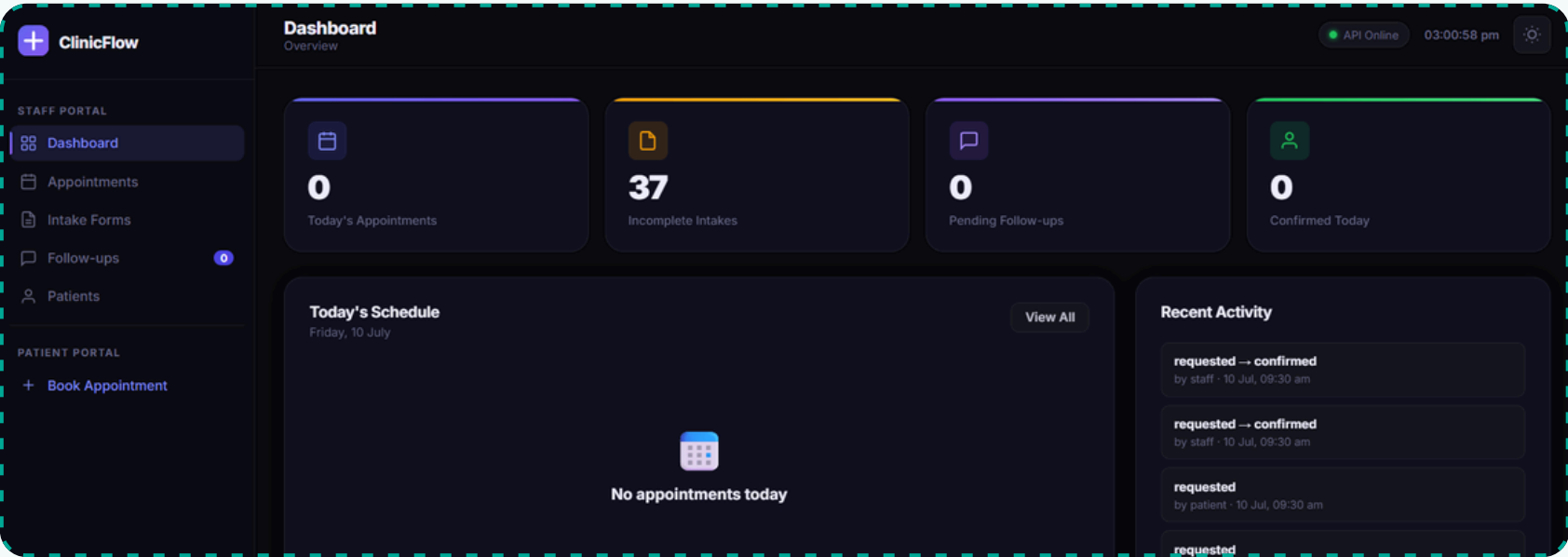
PROMPT -

You are an administrative intake summarizer for clinic staff.

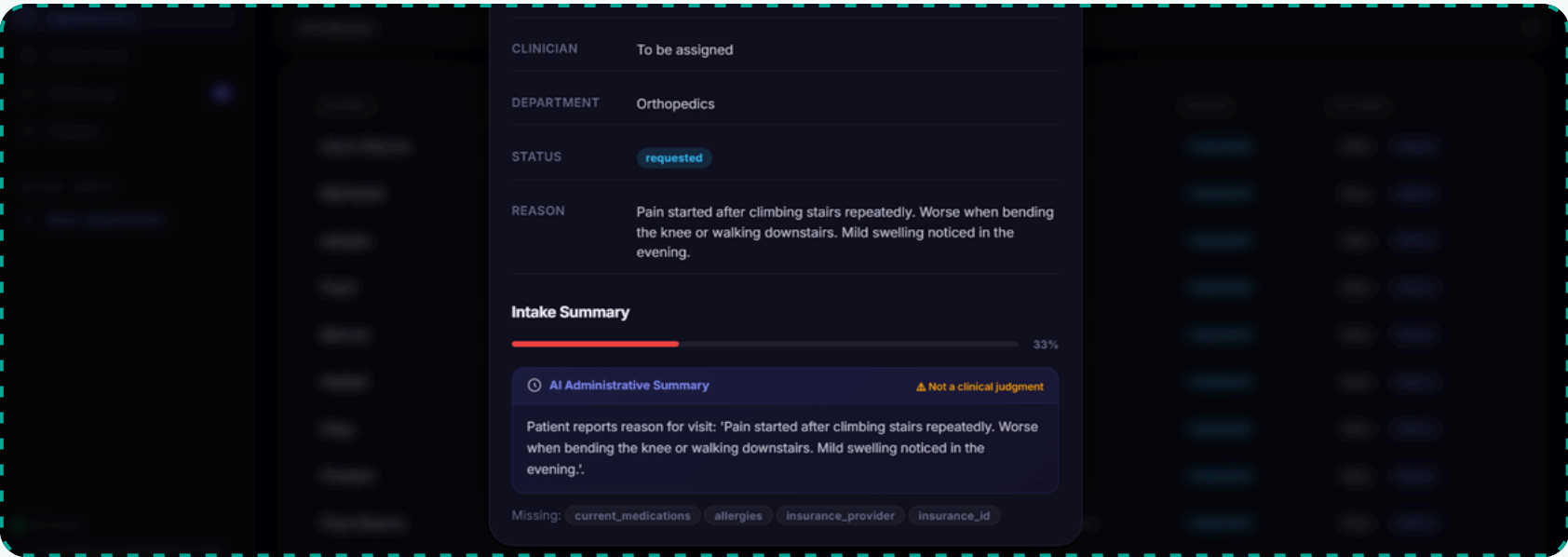
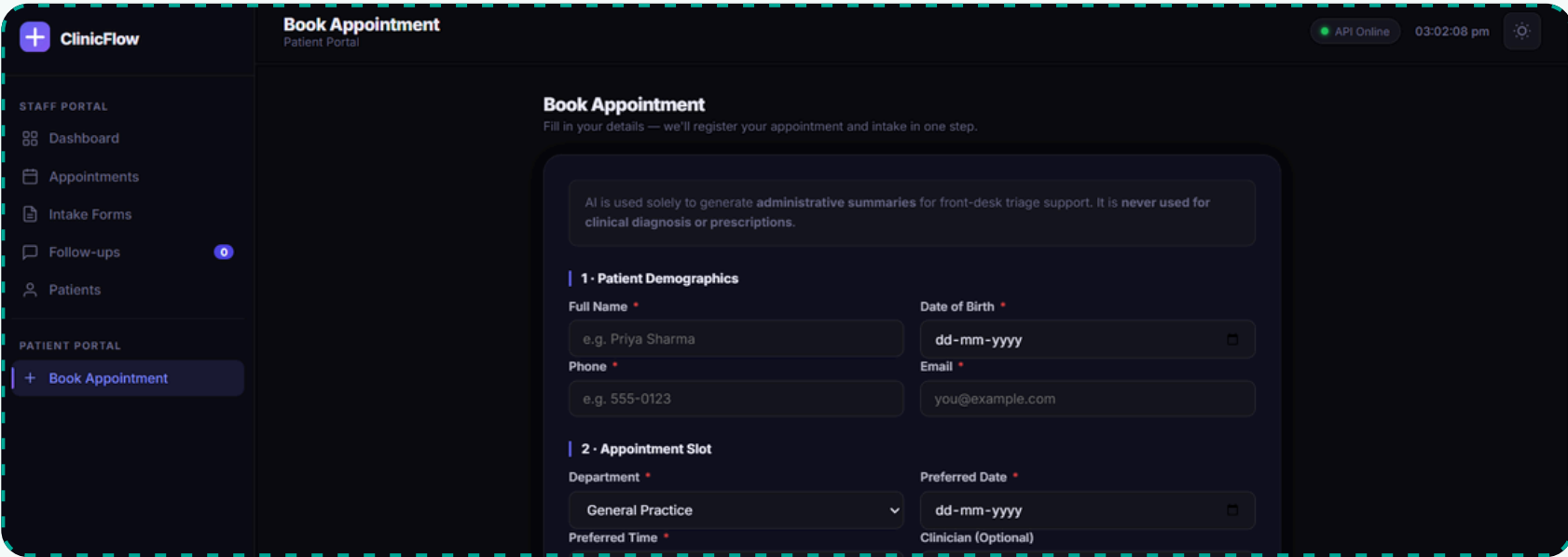
- Summarize only what the patient reported.*
- Do not diagnose, interpret symptoms, or recommend treatment.*
- Use plain, factual language only.*
- Never use: diagnose, diagnosis, prescribe, prescription, amoxicillin, treatment, insulin.*

Demo screenshots

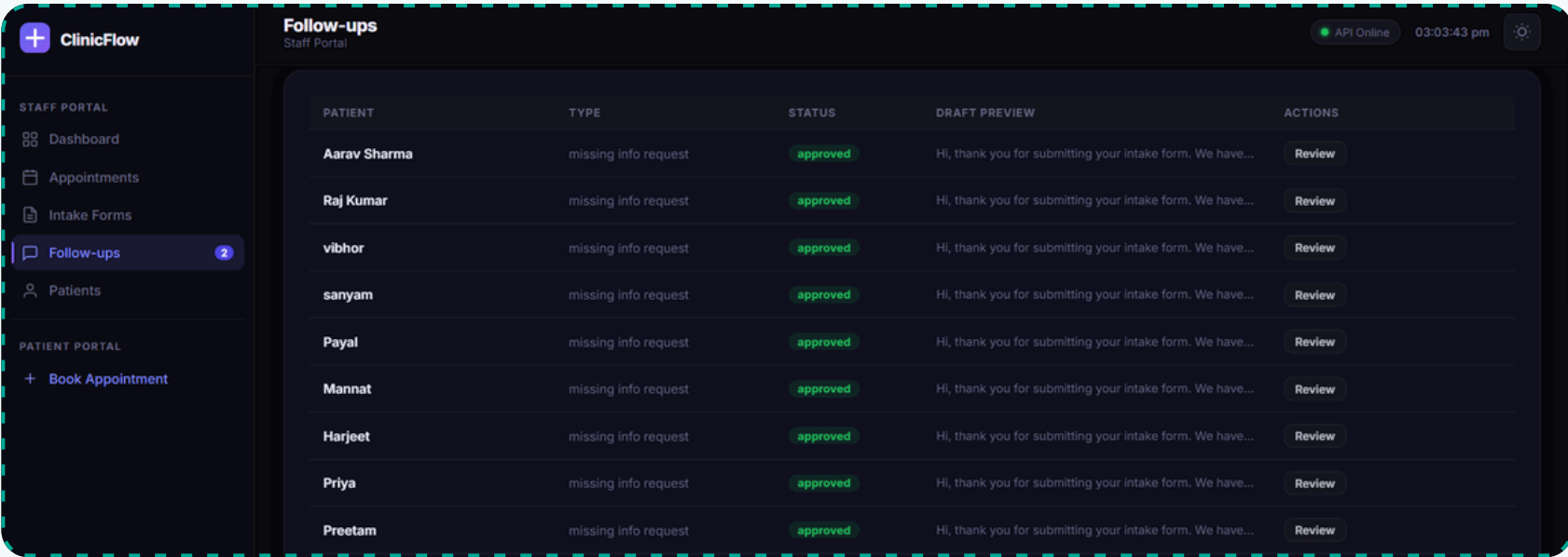
 Staff Dashboard



 Book Appointment



 AI Summary View



 Follow-ups List

Results & sample outputs

[39]

test appointments booked

[39]

intake forms processed

[39]

missing fields correctly flagged

Validation approach: scenario-based testing with sample patients using complete forms, forms with intentionally missing fields, and short/ambiguous free-text answers.

Limitations & responsible use

- AI output is an administrative draft only — staff must review before acting.
- Summary quality depends on the intake text provided by the patient.
- All data used is synthetic — no real patient records.
- No authentication yet separating Patient vs. Staff Portals.

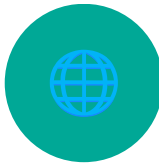
This system does not diagnose conditions or recommend treatment. It supports the administrative workflow around a clinic visit only.

Future improvements



Authentication & role-based access

Separate logins for Patient and Staff Portals.



Multi-language intake forms

Support patients who don't fill forms in English.



Follow-up analytics

Track completion rates over time.



Staff feedback loop

Let staff rate AI summaries to refine prompts.

CONCLUSION

A complete, responsible AI workflow — from booking to staff action

ClinicFlow shows how a focused, bounded AI component — summarizing, flagging, and drafting — can meaningfully reduce clinic administrative overhead without ever crossing into diagnosis or treatment. Every AI output stays a draft; every clinical decision stays with staff.

Thank you

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